

Policy Challenge 1: How can Switzerland contain healthcare costs?

Switzerland has pursued cost containment through changes to prices, payment systems, volumes, capacity, coverage and care organisation. Responsibility is dispersed: the Confederation regulates the KVG framework and benefits, cantons plan and co-finance care, and tariff partners negotiate much of provider payment.

Policies past and present

Payment, tariffs and capacity

- **SwissDRG / hospital financing (2012):** Case-based inpatient payment and revised hospital financing strengthened price and efficiency incentives; cantons remain central through hospital planning and co-financing. **FOPH: Hospital tariffs**
- **TARDOC + ambulatory flat rates (2026):** Replaced TARMED. The nationally uniform system combines fee-for-service with flat rates and is subject to cost-neutrality requirements. **FOPH: Ambulatory medical tariff**
- **Cantonal provider and capacity controls:** Cantons use hospital planning and, under federal rules, admission controls for physicians billing OKP to influence supply and volumes. **FOPH: Admission of service providers**
- **AL-Höchstgrenze:** The new ambulatory tariff system includes limits on billable medical-service tariff points per working day, illustrating volume control within negotiated tariffs. **FOPH: TARDOC and ambulatory flat rates**

Shift care to less costly settings

- **Ambulant vor stationär:** The Confederation specifies interventions that should generally be performed on an outpatient basis; cantons and providers implement the shift. **FOPH: Outpatient before inpatient**
- **EFAS (from 2028; long-term care from 2032):** Uniform financing removes the different financing shares for outpatient and inpatient care and is intended to improve incentives for outpatient and coordinated care. **FOPH: EFAS**

What should compulsory insurance pay for?

- **WZW and Health Technology Assessment:** OKP benefits must be effective, appropriate and cost-effective; HTA and reassessment can restrict or remove low-value services. **FOPH: HTA**
- **Benefit expansion can also increase expenditure:** The 2022 Anordnungsmodell gave psychological psychotherapists direct OKP billing rights on medical prescription. BAG reports OKP expenditure rising from CHF 528m in 2021 to CHF 922m in 2024, alongside improved access. **FOPH: Psychotherapy evaluation**
- **Low-value care:** Reimbursement restrictions, WZW reviews and initiatives such as smarter medicine target services whose benefits do not justify their costs. **FOPH: Cost Containment Package 2**

Pharmaceutical prices and volumes

- **Price regulation:** The Confederation sets prices for medicines on the Spezialitätenliste and periodically reviews them using therapeutic and international price comparisons. **FOPH: Medicines and tariffs**

- **Generics and biosimilars:** Price rules and differentiated patient cost sharing seek to increase use of cheaper alternatives. **FOPH: Medicines measures**
- **Cost Containment Package 2:** Cost-volume models introduce volume-related rebates for high-turnover medicines. The federal target is around CHF 350m in annual savings. **FOPH: Pharmaceutical measures**

Coordination and digitalisation

- **Managed care and integrated-care models:** Gatekeeping, physician networks, capitation and telemedicine aim to reduce avoidable utilisation. A Swiss study estimated efficiency effects of 21.2% for contracted capitated networks and 15.5% for contracted non-capitated networks; these are not system-wide savings estimates. **Reich et al. (2012)**
- **Digitalisation:** Electronic records, e-prescriptions, decision support, teleconsultations and automated workflows may reduce duplication and administrative costs. A McKinsey modelling study estimated a theoretical potential of CHF 8.2bn, mostly from provider productivity. **McKinsey: Digitisation in healthcare**

System-wide cost control

- **Cost-Containment Packages 1 and 2:** Federal reform packages combine measures on tariffs, volumes, benefits, pharmaceuticals and transparency. **FOPH: Package 2**
- **Cost Brake Initiative (2024) and cost targets:** Voters rejected the initiative; the indirect counterproposal instead introduced four-year cost and quality targets for the OKP. **FOPH: Cost and quality targets**
- **Round Table on Cost Containment:** Confederation, cantons, insurers, providers and industry negotiate measures that can be implemented relatively quickly. **FOPH: Round Table**

The recurring policy choices

- Lower prices or reduce volumes?
- Restrict supply or preserve access and patient choice?
- Pay for innovation or limit the benefit basket?
- Shift care to cheaper settings or reduce care altogether?
- Rely on competition and provider incentives or stronger state regulation?
- Control individual services or set system-wide cost targets?
- Contain costs now or invest in coordination and digitalisation for longer-term efficiency?

Policy Challenge 2: How should the costs of healthcare be distributed?

Swiss healthcare costs are distributed through insurance premiums, taxes and direct household payments. While the Confederation defines the basic KVG framework, the cantons have substantial responsibility for premium subsidies and healthcare financing. This has produced recurring conflicts over solidarity, individual responsibility and the division of costs between premium payers and taxpayers.

Policies past and present

Premium subsidies: how much redistribution?

- **Individual premium reductions (IPV):** Cantons implement premium subsidies and contribute their own funding alongside the federal contribution. This gives cantons considerable influence over the effective premium burden of lower- and middle-income households. [FOPH: Premium subsidies](#)
- **Vaud 10% model (2019):** Vaud introduced subsidies intended to limit the recognised premium burden to 10% of a household's relevant income, an important example of cantonal policy innovation. [Canton of Vaud](#)
- **Premium Relief Initiative (2024):** Proposed a nationwide 10% limit on the premium burden, financed through additional federal and cantonal subsidies. Voters rejected the initiative. [FOPH: Premium Relief Initiative](#)
- **Federal counterproposal (2026):** Instead of a uniform national ceiling, cantons must make a minimum contribution to premium subsidies and define a maximum acceptable premium burden relative to disposable income. This preserves substantial cantonal discretion. [FOPH: Counterproposal](#)

One insurer or competing insurers?

- **Social single health insurer initiative (2007):** Proposed replacing competing insurers with a single health insurer and setting premiums according to insured persons' economic capacity. Rejected by voters. [Swissvotes: 2007 vote](#)
- **Public health insurer initiative (2014):** Proposed one national public insurer with cantonal or intercantonal agencies. Unlike the 2007 initiative, it did not propose income-related premiums. Rejected by voters. [Swissvotes: 2014 vote](#)
- **New SP income-dependent premium initiative (launched 2025):** Proposes income-dependent premium rebates, financed by capped surcharges on higher incomes; children and young people under 18 would pay no OKP premiums. The existing choice of insurer, insurance model and deductible would remain. [SP Switzerland: initiative proposal](#)

How much should patients pay themselves?

- **Deductibles and coinsurance:** Federal policy determines how much patients pay before and alongside insurance. The standard adult deductible is CHF 300, followed by 10% coinsurance up to an annual ceiling. Optional higher deductibles reduce premiums but increase individual financial exposure. [FOPH: Premiums and cost sharing](#)
- **Planned increase in the minimum deductible (2026 proposal):** Following Parliament's adoption of Motion 24.3636, the Federal Council opened consultation on raising the current CHF 300 minimum deductible and introducing a mechanism for moderate periodic adjustment. Children would remain exempt. [FOPH: adjustment of the deductible](#)

- **Differentiated cost sharing for medicines:** Higher cost sharing for certain more expensive medicines is used to encourage substitution towards cheaper alternatives. **FOPH: Medicines measures**

Premiums or taxes?

- **Hospital financing (since 2012):** Cantons co-finance inpatient hospital treatment from taxation, whereas outpatient OKP services have largely been financed through insurance premiums. This creates different financing incentives depending on where treatment occurs. **FOPH: Hospital tariffs**
- **EFAS (from 2028):** Uniform financing will apply the same financing division to outpatient and inpatient care: cantons finance at least 26.9% and insurers at most 73.1% of net costs. Long-term care follows in 2032. EFAS therefore also redistributes financing between taxpayers and premium payers. **FOPH: EFAS**

Who bears health risks?

- **Risk equalisation:** Transfers between insurers compensate for differences in the health risks of their insured populations. The system has repeatedly been strengthened, including the introduction of pharmaceutical cost groups (PCGs) in 2020. Unlike premium subsidies, this redistributes according to health risk rather than income. **FOPH: Risk equalisation**

The recurring policy choices

- Premiums or taxes?
- National redistribution or cantonal autonomy?
- More solidarity or more individual cost sharing?
- Competing insurers or a public single insurer?
- Income-independent premiums or contributions based more strongly on ability to pay?

Policy Challenge 3: How can Switzerland secure access to health-care with a constrained workforce?

Responsibility for securing healthcare provision is fragmented. The Confederation regulates professions, insurance and parts of education policy; cantons have major responsibilities for healthcare provision, planning and provider admission; employers determine many working conditions; and municipalities can intervene where local services are threatened. The challenge is therefore not simply to train more professionals, but to get the right workforce, with the right skills, to the places where it is needed.

Policies past and present

Train more

- **Expansion of medical education:** The federal Special Programme Human Medicine (2017–2020) provided CHF 100 million to expand training capacity, with a target of around 1,350 medical degrees annually by 2025. [SERI: Special Programme Human Medicine](#)
- **Nursing Initiative (accepted 2021):** Created a constitutional mandate to strengthen nursing. The first implementation package, in force since 2024, focuses particularly on expanding nursing education and training, with financing shared by the Confederation and cantons. [FOPH: Nursing Initiative](#)

Retain the workforce

- **Working conditions in nursing:** The second implementation stage of the Nursing Initiative proposes federal rules concerning working conditions, including working time and scheduling, alongside measures to strengthen professional development. [FOPH: Nursing Initiative implementation](#)
- **Working conditions in medicine:** Long working hours, duty planning, administrative burden and opportunities for part-time work influence the attractiveness of medical careers. The association of resident and senior physicians (vsao) advocates better working conditions, including compliance with labour law and the 42+4-hour working week. Hospitals and other employers are central actors alongside government. [vsao: Working conditions](#)

Control where providers enter the system

- **Physician admission restrictions / Höchstzahlen:** Cantons can restrict the number of physicians permitted to bill the OKP in particular specialties and regions under the federal KVG framework. The instrument allows geographically and specialty-specific steering of supply. [FOPH: Maximum numbers for physicians](#)
- **This creates a direct tension with workforce policy:** Cantons may seek to prevent oversupply in some specialties or regions while simultaneously trying to attract professionals to underserved areas.

Attract professionals to underserved areas

- **Praxisassistentz:** Cantonal programmes subsidise postgraduate training positions in primary-care practices, creating a pathway into general practice and local primary care. [Example: Canton of Bern](#)
- **Cantonal and municipal support:** Public authorities can support group practices, health centres and regional care networks where local provision is threatened. The federal Agenda

Grundversorgung explicitly includes measures to strengthen primary care in rural and peripheral regions in cooperation with cantonal, municipal and regional actors. **FOPH: Agenda Grundversorgung implementation**

Use the workforce differently

- **APNs and expanded nursing roles:** Current reforms seek to use advanced nursing competencies more extensively, including greater professional autonomy. **FOPH: Nursing Initiative**
- **Task sharing and interprofessional primary care:** Group practices, health centres and regional networks can distribute responsibilities across physicians, nurses/APNs, pharmacists and other professionals rather than relying primarily on the individual physician. This is also a priority of the Agenda Grundversorgung. **FOPH: Agenda Grundversorgung implementation**

Change financial incentives and models of care

- **TARDOC and ambulatory flat rates (2026):** The new tariff system replaced TARMED on 1 January 2026. TARDOC introduced a dedicated chapter for family medicine and paediatrics, intended to improve the remuneration of primary care. **FOPH: Ambulatory medical tariff**
- **Controversy over primary-care remuneration:** In September 2026, the association of family doctors and paediatricians (mfe) reported that the intended improvement had not materialised: family medicine had seen no overall upgrading and, in some cantons, even a deterioration. It warned that the tariff imbalance between primary and specialist care could widen. The FMH acknowledged these concerns but cautioned that sufficiently robust data on the effects across specialties were not yet available. **SRF: Primary-care criticism of TARDOC; FMH: Statement, 3 September 2026**
- **Tariff revision and cost neutrality:** The tariff partners are responsible for further developing TARDOC. The federal Agenda Grundversorgung includes specific monitoring of its effects on primary care. A central policy question is how to improve remuneration for primary care while maintaining the cost-neutrality requirements of the tariff transition. **FOPH: Tariff framework; FOPH: Agenda Grundversorgung**

The recurring policy choices

- Train more or retain more?
- Allow more providers or restrict supply to control costs?
- National solutions or targeted cantonal and regional interventions?
- Subsidise professionals in underserved areas or reorganise service delivery?
- More doctors and nurses or redistribute tasks across professions?
- Improve working conditions even if this increases costs?
- Strengthen primary-care remuneration even if this requires redistribution within the tariff system?

Policy Challenge 4: How can Switzerland strengthen public health?

Public health policy acts before, alongside and beyond healthcare. Responsibility is dispersed across the Confederation, cantons, municipalities, healthcare actors and other policy sectors. The central question is how far government should rely on information and individual responsibility, and how far it should regulate products, prices and the environments that shape health.

Policies past and present

Inform and enable

- **NCD Strategy (2017–2028):** Developed jointly by the Confederation, cantons and Health Promotion Switzerland, it combines population-based prevention, prevention in healthcare, workplace health promotion and cross-cutting measures. Current priorities include mental health, tobacco/nicotine and overweight/obesity. [FOPH: NCD Strategy](#)
- **Health2030 (2020–2030):** The Federal Council's overarching health policy strategy extends beyond healthcare delivery. Its objectives include preventing non-communicable diseases, promoting child and adolescent health, reducing environmental health risks and fostering healthy working environments. It provides strategic direction but does not itself establish a comprehensive federal prevention framework. [FOPH: Health2030](#)

Regulate or incentivise healthier consumption

- **Tobacco regulation:** The Tobacco Products Act has applied since October 2024. The 2022 Tobacco Advertising Initiative led to further restrictions intended to protect children and young people; the implementing revision is expected to enter into force in 2027. [FOPH: Tobacco Products Act](#)
- **Sugar reduction:** Swiss nutrition policy has relied substantially on voluntary agreements with food and beverage producers. The Declaration of Milan was renewed in 2025, with participating companies committing to further sugar reductions through 2028. Taxes on sugar-sweetened beverages represent a contrasting, price-based instrument, but are not current federal policy. [FSVO: Sugar reduction](#)

Coordinate and finance prevention across a federal system

- **Low prevention expenditure:** Despite high overall healthcare spending, Switzerland allocates only 1.9% of current health expenditure to preventive care, compared with an OECD average of 3.4%. This places Switzerland near the bottom of the OECD range. The indicator captures expenditure classified as preventive care, not every prevention activity undertaken across government and other sectors. [OECD: Health at a Glance 2025, Switzerland](#)
- **Failed Prevention Act (2009–2012):** The proposed federal law sought stronger coordination of prevention and health promotion. It ultimately failed in Parliament, illustrating the difficulty of creating a stronger national framework in a policy field with important cantonal responsibilities. [Swiss Parliament: Prevention Act](#)
- **Current approach:** National strategies coordinate prevention within existing responsibilities. The NCD Strategy is jointly governed by FOPH, GDK and Health Promotion Switzerland, while Health2030 provides broader strategic direction. [FOPH: NCD Strategy](#)
- **Federal Health Act proposal (SAMW):** The Swiss Academy of Medical Sciences proposes a new constitutional health article and a Federal Health Act as a broader framework for health policy, emphasising One Health and Health in All Policies. This is an SAMW proposal, not current federal government policy. [SAMW: Federal Health Act proposal](#)

Address addiction and mental health

- **National Addiction Strategy (2017–2028):** Combines prevention, treatment and counselling, harm reduction/risk minimisation, and regulation/enforcement, extending the established Swiss four-pillar approach across different forms of addiction. **FOPH: National Addiction Strategy**
- **New umbrella prevention strategy (from 2029):** The Confederation, cantons and Health Promotion Switzerland are developing a joint strategy combining NCD prevention, addiction and prevention of mental illness. It is scheduled for approval in 2027, with implementation beginning in 2029. **FOPH: Future prevention strategy**

Protect the population from collective health threats

- **Epidemics Act:** Communicable-disease control requires cooperation between Confederation and cantons. The current revision draws lessons from Covid-19 and addresses surveillance, crisis coordination, medical supplies, vaccination access and antimicrobial resistance. **FOPH: Epidemics Act revision**
- Communicable disease provides a distinct rationale for public-health intervention because individual decisions can create risks for others and effective prevention requires surveillance and coordinated collective action. **FOPH: Epidemics Act**

Act beyond the healthcare system

- Health is shaped by living and working conditions as well as healthcare. The NCD Strategy therefore includes workplace health promotion and health-promoting environments, while the SAMW proposal explicitly advances Health in All Policies and One Health. **FOPH: NCD Strategy**

The recurring policy choices

- Individual responsibility or collective responsibility?
- Information, voluntary agreements, taxes or regulation?
- National coordination or cantonal autonomy?
- Treat disease or invest earlier in prevention?
- Health policy inside or outside the healthcare system?
- Does Switzerland need a stronger federal framework for health?

Policy Challenge 5: Can Swiss healthcare become simpler without creating more regulation?

Administrative complexity has many sources: federal and cantonal regulation, insurer controls, provider organisations, fragmented information systems and legal safeguards. State activity has also expanded as policymakers respond to costs, quality, access and data problems. Each intervention may pursue a legitimate goal while adding new reporting, approval or monitoring requirements.

Policies and conflicts

More steering can mean more administration

- The regulatory framework has expanded over time. A CSS Dialog compilation reports that between 2000 and 2022 the KVG grew from 40 to 98 pages, while KVV and KLV together grew from 122 to 190 pages. Page counts are only a rough indicator, but they illustrate the increasing density of regulation. [CSS Dialog: Who creates bureaucracy?](#)
- Recent cost-containment reforms illustrate the tension. Package 2 alone contains 16 measures and requires implementing rules on benefits, medicines, prices and cost-volume models. Such measures may contain expenditure or improve access, but they can also require additional data, assessments and controls. [FOPH: Cost-Containment Package 2](#)
- Provider organisations such as FMH and SGAIM increasingly criticise 'micro-regulation' and administrative burden. These are stakeholder positions, but they highlight an important policy question: when does additional state steering cease to be worth its implementation costs? [SGAIM: Papiertiger](#)

Providers and insurers both generate and implement controls

- SGAIM's 2026 Papiertiger campaign, based on responses from more than 1,800 health professionals, highlights repeated data entry, incompatible IT systems, insurer queries, prior authorisations and medication reconciliation as sources of avoidable work. [SGAIM: Papiertiger](#)
- Insurers request documentation and prior approval to check entitlement, effectiveness, appropriateness and cost-effectiveness and to control expenditure. Providers experience these controls as administration; insurers themselves must comply with KVG rules and supervision. Responsibility for bureaucracy therefore often runs through a chain of actors rather than having a single source. [FOPH: Administrative burden](#)

Medicines: faster access, cost control and administrative rules

- Exceptional and off-label reimbursement under Articles 71a–71d KVV requires individual assessment by insurers. A federal evaluation found that all stakeholder groups considered the administrative burden too high. Subsequent reforms introduced more standardised benefit assessment in an attempt to make decisions more consistent. [FOPH: Individual-case reimbursement](#)
- Package 2 now adds new pharmaceutical instruments including cost-volume models, price models, reimbursement from Swissmedic authorisation ('Day 0') and differentiated WZW assessment. The 2026 KVV/KLV consultation shows the trade-off clearly: faster access and tighter price/cost control require detailed rules for manufacturers, insurers and authorities. [FOPH: 2026 medicines implementation](#)

Law and courts set limits on simplification

- Health-insurance administration is also shaped by law and Federal Supreme Court jurisprudence. Courts clarify what counts as an insured entitlement, how WZW and economic-efficiency requirements must be applied, and which procedures insurers must follow. These rulings can require more careful documentation and individual assessment, but they also constrain arbitrary decisions and protect patients and providers. [Federal Supreme Court](#)
- **The general dilemma is therefore not 'bureaucracy versus no bureaucracy':** simpler procedures must still provide enough evidence for equal treatment, reimbursement decisions, cost control and judicial review.

Digitalisation: remove duplication or digitise it?

- DigiSanté (2025–2034) aims to create interoperable data flows so that information can be exchanged and reused across the health system. The FOPH explicitly identifies repeated collection of the same data and insufficient interoperability as current weaknesses. [FOPH: DigiSanté](#)
- **Electronic health dossier:** the planned E-GD is intended to replace the current EPD and establish more systematic use of electronic health information. Common standards and infrastructure can simplify information exchange, but building them itself requires legislation, governance, technical standards and investment. [FOPH: Electronic health dossier](#)
- **Once Only:** In February 2026 the Federal Council sent Parliament a KVG amendment under which administrative health data should be collected once and reused by authorised actors. It is a particularly clear example of simplification through better infrastructure and regulation rather than deregulation. [FOPH: Once Only](#)

Simplify structures as well as paperwork

- EFAS removes the historic difference in financing between outpatient and inpatient care, and later long-term care. It simplifies one major structural divide in the system, although implementation itself requires new rules, data flows and coordination. [FOPH: EFAS](#)
- The FOPH has also launched a dedicated project to measure administrative burden before developing targeted measures. Its literature review concludes that Switzerland still lacks systematic evidence on the amount, distribution and causes of administrative work. [FOPH: Administrative burden](#)

The recurring policy choices

- Less regulation or better regulation?
- More state steering or more room for self-regulation?
- Cost control or administrative relief?
- Trust providers or verify claims?
- Collect less data or collect it once and reuse it?
- Digitalisation as simplification or another layer of complexity?
- Simpler procedures or stronger legal safeguards?
- Simpler for whom: patients, providers, insurers or government?